A/PROF CHAMARA BASNAYAKE

MBBS, PhD, FRACP

Gastroenterologist & Endoscopist

PN 421926PK

FOCUS GASTROENTEROLOGY

20 May 2026

Dr Poovadan Sudhakaran

Dear Dr Sudhakaran,

Re: Ms Anne-Marie Avramopoulos
DOB: 16/05/1980 Phone: 0417563910
Address: 12 O'Donnell Street, RESERVOIR, VIC, 3073
Email: annafavram@hotmail.com

Summary

- **Persistent left-sided abdominal pain and symptoms of obstructed defaecation.**
- **Possible mild diverticulitis or functional bowel disturbance.**
- **For gastroscopy and colonoscopy.**
- **No action required by you.**

I had the pleasure of reviewing Anne Marie in the rooms today. She presents with a one-month history of abdominal pain, which initially began in the right lower quadrant and pubic region before localising to the left side and referring to her back.

Anne Marie has seen you previously regarding these symptoms, where a urinary tract infection and other infections were excluded. She was trialled on doxycycline and a second course of antibiotics, though neither resolved the discomfort. A subsequent presentation to the Epworth Emergency Department led to a pelvic ultrasound, which identified a uterine polyp and adenomyosis; however, her obstetrician does not believe these findings explain the current nature of her pain.

Clinically, she describes symptoms of tenesmus and significant constipation. She has been using Microlax enemas and macrogol with some success, though she notes episodes of abdominal spasming and significant nausea at the peak of her pain. There has been no report of haematochezia or mucus. On examination today, she was tender in the left iliac fossa, though there were no peritonitic features or concerning masses.

Impression and Plan

I suspect this may represent an episode of mild diverticulitis, though I am mindful of her history of visceral hypersensitivity. While she had a colonoscopy three years ago, her symptoms are persisting and I believe a repeat evaluation is now indicated to ensure no new pathology has developed in that interval. I have recommended a combined gastroscopy and colonoscopy. I have opted to include the gastroscopy as her previous procedure was performed while she was on a high dose of esomeprazole and nizatidine, which may have masked acid-related changes. Performing the procedure now, while her use of these medications is sporadic, may provide a clearer clinical picture. If these investigations are unremarkable, we will then proceed to a CT scan to further evaluate the source of her pain, including the possibility of renal calculi, although her clinical presentation is not classically suggestive of this. I have advised her to prioritise stool softeners such as macrogol over regular stimulant laxatives. I will review her following these procedures.

All appointments and enquires

Suite 201, 100 Victoria Parade
East Melbourne VIC 3002
Ph: 03 9650 7917
Fax: 03 9650 7910
Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations

Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

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Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

CC: Dr Amanda Richards

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